Royal Devon University Healthcare NHS Foundation Trust

Cardiology Day Case Unit (CDCU) Wonford site, Elective Angiography Pre-Assessment	
Post holder responsible for Procedural Document	Clinical Nurse Manager, CDCU
Authors of Standard Operating Procedure	James Chapman, Clinical Nurse Manager
Division/Department responsible for Procedural Document	Medicine / Cardiology
Contact details	James Chapman – 01392 408138
Date of original standard operating procedure	September 2025
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Approving body and date approved	Cardiology Specialty Governance Group, 24 September 2025
Expiry date	September 2028
Date document becomes live	Date document is published, circulated & put onto the Trust intranet

Please *specify* standard/criterion numbers and tick ✓ other boxes as appropriate

Monitoring Information		**Strategic Directions – Key Milestones**	
Patient Experience	✓	Maintain Operational Service Delivery	✓
Assurance Framework	✓	Integrated Community Pathways	
Monitor/Finance/Performance	✓	Develop Acute Services	
CQC Fundamental Standards Regulations No:		Delivery of Care Closer to Home	
		Infection Control	
Other *(please specify)*:			
Note: This document has been assessed for any equality, diversity or human rights implications			

Controlled document
This document has been created following the Royal Devon University Healthcare NHS Foundation Trust Policy on Procedural Documents. It should not be altered in any way without the express permission of the author or their representative.

Full History		Status: FINAL	
Version	Date	Authors (Title not name)	Reason
1.0	22.05.2025	Clinical Nurse Manager; Clinical Pathway Transformation Manager	New Document

Associated Trust Policies/Procedural documents:	Infection Prevention and Control Policy
Key Words:	
In consultation with and date: Sarah Dale, Clinical Matron, Cardiology, Cardiology Specialty Governance Group, Medical Services Divisional Governance Group, Divisional Management Team,	
Contact for Review:	James Chapman, Clinical Nurse Manager

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1. INTRODUCTION

1.1 The Trust has a Cardiology Day Case Unit (CDCU) on the Wonford site undertaking elective procedures on patients as follows:

2. PURPOSE

2.1 To provide all clinical, managerial and administrative staff working within the Trust and clear guidance.

- This Standard Operating Procedure (SOP) outlines the Angiography Pre-Assessment Process for Elective Cardiology Day Case Patients

3. Start Of Pre-assessment Process

3.1 Identify patients for pre-assessment once listed using My reports or snap board on EPIC.

3.2 Contact patient via telephone number provided on EPIC for the individual patient. If unable to contact them leave a voice message, giving the pre-assessment number 01392 408138 asking for a return call. Patients to be contacted three times and if no contact made contact booking team to alert unable to make contact.

4. Pre-Assessment

4.1 Use Generic Pre-Assessment Proforma as guide. See appendix 1

4.2 Identify Procedure to be performed.
1. Diagnostic angiography (LHC) only
2. Angiography +/- diagnostic tests (PWS/IVUS)
3. PCI, or "?proceed"

4.3 Identify patient has been listed in correct lab

5. Individual pre-assessment questions per Procedure.

5.1 PCI patients – determine if patient has been prescribed antiplatelet (clopidogrel) - loaded or takes regularly. Determine if the patient has been prescribed Aspirin - loaded or takes regularly. Check if patient has any allergies to these medications. Inform Consultant if allergies are identified via email or EPIC messenger.

5.2 Request consultant to order Clopidogrel and Aspirin if required for day case nurse to administer on day of procedure.

6. Overnight Stay

6.1. See SOP for overnight stays/criteria

6.2 Patients undergoing certain procedures will require an overnight stay on the cardiology ward and this will have been planned prior to their admission. In the event that a patient needs an overnight bed that has not been pre-planned the recovery staff will need to escalate this to the senior nurse on duty who will contact the site management team to request an overnight bed for the patient.

7. Infection control

7.1 Identify using EPIC any infection control issues including CPE, MRSA, VRE, ESBL.

7.2 Clarify with patient if they have been in any hospital outside Devon, Cornwall, or Somerset and inform booking team if patient alerts that this has occurred to highlight on lab sheets.

7.3 Clearly document any requirement of use of side room on day of procedure.

7.4 Inform Cath lab co-ordinator and CDCU manager of any infection control issues.

8. CONCLUSION

This SOP is intended to ensure that patients receive high-quality Pre-Assessment prior to procedure taking place

9. ARCHIVING ARRANGEMENTS

The original of this SOP will remain with the author James Chapman, Clinical Nurse Manager, CDCU. An electronic copy will be maintained on the Trust Acute Medicine Hub page. Archived electronic copies will be stored on the Trust's “archived policies” shared drive, and will be held indefinitely. A paper copy (where one exists) will be retained for 10 years.

10. PROCESS FOR MONITORING COMPLIANCE WITH AND EFFECTIVENESS OF THE STANDARD OPERATING PROCEDURE

To evidence compliance with this policy, the following elements will be monitored:

What areas need to be monitored?	How will this be evidenced?	Where will this be reported and by whom?
Staff competency and ongoing service provision	Initial sign off Yearly competency assessment by line manager	Record of competency in staff file Yearly competency assessment by line manager
Patient waiting times for off-loading or number of breaches	Monthly KPI report Datix Reporting	Specialty Meetings (Governance and Divisional)

Appendix 1: Pre-Assessment proforma as entered on EPIC

Check patient details correct on EPIC

Check that the patient is aware what procedure they are expecting and have been booked for.

Check appointment date/time (ensure patient understands time listed on letter is arrival time)? Inform patient of recovery times for their procedure and potential wait times after being admitted.

Is PMH correct on EPIC

Understands NBM instructions? – Refer to appendix 2.

Review the list of medications on the system with the patient and check these are correct? Add new medications and clearly note on home meds the ones the patient reports no longer taking.

Angioplasty, query angioplasty specific medication review.
Review antianginals – Patients should be on a beta blocker, calcium channel blocker and a nitrate – if not on one of all these class of drugs inform consultant undertaking the procedure to ensure patient is suitable to proceed to angioplasty.

Any previous DVT / PE?

Any allergies? Any contrast/iodine allergies? Any medication allergies?

Any medication changes/stop prior to procedure?

Any PPM/ICD/Valve etc?

If patient has had valve surgery, enquire which valve and query metallic and on warfarin.

Any symptoms (palpitations, chest pain)? If patient reports no symptoms for an extended period of time – email consultant to clarify if the patient still requires procedure.

Who is collecting/dropping off? Who is staying with patient overnight?

Any sleep apnoea? Can they lay flat?

Any Infection control issues (hospital outside Devon/Cornwall etc)? – Raise an alert if issues identified to ensure side room etc.

Latest Echo/CMRI?

Do they drive? Give driving instructions

Any plans to travel abroad?

Explain wait time & recovery time for procedure.

Any other issues?

APPENDIX 2: Pre-operative fasting guidelines

Clinical Guideline for: **Pre-Operative Fasting**

1. Introduction

The RD&E guideline for fasting is based on recommendations published by the European Society of Anaesthesiology (Published January 2022) and updated to incorporate the growing body of evidence to support the safety and benefits of drinking water in the peri-operative period.

2. Pre-Operative Fasting Guideline

2.1 Prior to surgery the recommended fasting periods are:

6 Hours	Solid food, or milk
4 Hours	Formula milk (under 1 year olds)
3 Hours	Breast milk
2 Hours	Clear fluids *(including tea and coffee with small amounts of milk < 2 tbsp)* Pre-op carbohydrate drinks
Unrestricted	Water or squash (still and unflavoured), tea or coffee without milk, non pulp juices. **Sip until send**
• Patients should be permitted to drink sensible amounts of water or squash in order to remain hydrated until called to theatre **Sip until send** • Patients should not have their surgery cancelled or delayed just because they are chewing gum or sucking a boiled sweet prior to induction although this is discouraged from 2 hours before surgery please.	

2.2 Fasting recommendations by type of anaesthesia:

GA	All patients should be fasted according to the above guidelines.
Regional block/ Spinal	
Sedation	All patients would normally be fasted *
Local anaesthetic only	No fasting is required and patients may eat a normal diet **

* There are some specific exceptions to this, e.g. dental surgery, where departmental policies exist.

** If listed to be seen by anaesthetist, may require fasting – see local policies.

2.3 Patients undergoing bowel preparation, the elderly, the sick, children and breastfeeding mothers may require intravenous fluids prior to surgery if fasting is prolonged.

2.4 Oral medications should not normally be omitted due to the patient being fasted for surgery

2.5 Some patients, such as those with bowel obstruction may still be 'Nil By Mouth' if requested by their surgical team. These patients should usually be prescribed IV fluids.

3. Contact Information

3.1 Any queries regarding pre-operative fasting should be directed to the anaesthetist involved, or via the Department of Anaesthesia.

4. Publication Details

Author of Clinical Guideline	Dr Pippa Dix, Consultant Anaesthetist Dr Marc Ridgeway, Specialist Anaesthetist
Division responsible for Clinical Guideline	Anaesthetics, Surgical Services Division
Contact details	x2474 philippa.dix@nhs.net
Version number	5.0
Replaces version number	4.1
Date written	August 2007 (reviewed: August 2010, March 2013, October 2016, July 2019, November 2020, November 2023)
Approving body and date approved	Anaesthetics Governance Group: 09/01/2024 (chair's approval)
Review Date (3-6 months prior to expiry date)	01/10/2026
Expiry date	09/01/2027
Date Document becomes live	09/01/2024

References:

European Society of Anaesthetics: Pre-operative fasting in adults and children.

http://www.aagbi.org/sites/default/files/Perioperative_fasting_in_adults_and_children__.4.pdf

Frickholm P, et al; European Journal of Anaesthesiology: Pre-operative Fasting in Children. January 2022 – p4-25

McCracken, Graham C.; Montgomery, Jane Postoperative nausea and vomiting after unrestricted clear fluids before day surgery, European Journal of Anaesthesiology: May 2018 - Volume 35 - Issue 5 - p 337-342

APPENDIX 3: EQUALITY IMPACT ASSESSMENT TOOL

Name of document	CDCU Ablation Pre-Assessment SOP
Division/Directorate and service area	Medicine – Eastern services
Name, job title and contact details of person completing the assessment	James Chapman Clinical Nurse Manager
Date completed:	22.05.2025

The purpose of this tool is to:

- **Identify** the equality issues related to a policy, procedure or strategy.
- **Summarise the work done** during the development of the document to reduce negative impacts or to maximise benefit.
- **Highlight unresolved issues** with the policy/procedure/strategy which cannot be removed but which will be monitored, and set out how this will be done.

1. **What is the main purpose of this document?**
   To provide all clinical, managerial and administrative staff working within the Trust clear guidance on:

   - Recovery of patients in the 6 bedded recovery area in the CDCU.
   - Recovery of patients in the 12 recliners chairs in the radial lounge in the CDCU.

2. **Who does it mainly affect?** *(Please insert an "x" as appropriate:)*
   Carers ☐ Staff ☒ Patients ☒ Other (please specify) ambulance crew staff

3. **Who might the policy have a 'differential' effect on, considering the "protected characteristics" below?** (By *differential* we mean, for example that a policy may have a noticeably more positive or negative impact on a particular group e.g. it may be more beneficial for women than for men)
   Please insert an "x" in the appropriate box *(x)*

Protected characteristic	Relevant	Not relevant
Age	X	☐
Disability	X	☐
Sex - *including: Transgender, and Pregnancy / Maternity*	X	☐
Race	☐	X
Religion / belief	☐	X
Sexual orientation – *including: Marriage / Civil Partnership*	☐	X

4. **Apart from those with protected characteristics, which other groups in society might this document be particularly relevant to…** (e.g. those affected by homelessness, bariatric patients, end of life patients, those with carers etc.)?

Bariatric patients, end of life patients and patients who do not have capacity.

5. Do you think the document meets our human rights obligations? ☒

Feel free to expand on any human rights considerations in question 6 below.

A quick guide to human rights:
• ***Fairness –*** *how have you made sure it treats everyone justly?* • ***Respect –*** *how have you made sure it respects everyone as a person?* • ***Equality –*** *how does it give everyone an equal chance to get whatever it is offering?* • ***Dignity –*** *have you made sure it treats everyone with dignity?* • ***Autonomy –*** *Does it enable people to make decisions for themselves?*

6. Looking back at questions 3, 4 and 5, can you summarise what has been done during the production of this document and your consultation process to support our equality / human rights / inclusion commitments?

This is a new document that has been created due to the patients coming into the cardiology day case unit rather than the ward for their procedure to ensure patients are regularly assessed and monitored and safely progressed through the unit to improve flow though the unit and the Trust. There are no equality or human rights concerns in the creation of this document.

7. If you have noted any 'missed opportunities', or perhaps noted that there remains some concern about a potentially negative impact please note this below and how this will be monitored/addressed.

"Protected characteristic":	None
Issue:	n/a
How is this going to be monitored/ addressed in the future:	n/a
Group that will be responsible for ensuring this carried out:	n/a

APPENDIX 4: COMMUNICATION PLAN

The following action plan will be enacted once the document has gone live.

Staff groups that need to have knowledge of the SOP	Clinical Nurse Manager Clinical Matron Nursing Staff Medical Staff Reception Staff On Call Managers Site Management Team
The key changes if a revised document	New document
The key objectives	To provide all clinical, managerial and administrative staff working within the Trust and ambulance crew staff clear guidance on: •
How new staff will be made aware of the procedure/guideline and manager action	• Induction process. • Cascade from line manager. • Team meetings and regular means of communications.
Specific Issues to be raised with staff	Staff will be made aware of Generic Pre-assessment questions
Training available to staff	Support available during induction and daily clinical practice from the clinical nurse manager, clinical matron and their line manager. It is also included in the bespoke triage training session.
Any other requirements	n/a
Issues following Equality Impact Assessment (if any)	Patients will be assessed and monitored regularly to determine their clinical priority and any protected characteristics will be considered.
Location of hard / electronic copy of the document etc.	The hard copy of this SOP will remain with the author James Chapman, Clinical Nurse Manager, CDCU. An electronic copy will be maintained on the Trust Acute Medicine Hub page. Archived electronic copies will be stored on the Trust's “archived policies” shared drive, and will be held indefinitely. A paper copy (where one exists) will be retained for 10 years.